

PulseSync Medical - Implant Operation Note

Patient Name	Mark Smith	DOB	1976-05-22
Hospital	PulseSync Medical Specialty Hospital	Patient ID	PSM491002

Procedure Information

Procedure	Dual Chamber Pacemaker Implantation
Date of Implant	2025-06-20
Implant Site	Right pectoral

Device Details

Device Model	Dual Chamber MRI SureScan Pacemaker
Device Serial	PSM482917
Indication	Symptomatic bradycardia due to suspected sinus node dysfunction

Lead Details

Atrial Lead Model	PS-ALD 4574
Atrial Lead Serial	ALD4574-219384
Atrial Lead Position	Right Atrial Appendage
Ventricular Lead Model	PS-VLD 5076
Ventricular Lead Serial	VLD5076-773512
Ventricular Lead Position	Right Ventricular Apex

Intraoperative Measurements

RA Sensing (P-wave)	3.8 mV
RA Threshold	0.8 V @ 0.4 ms
RA Lead Impedance	520 Ω
RV Sensing (R-wave)	9.6 mV
RV Threshold	0.7 V @ 0.4 ms
RV Lead Impedance	610 Ω
Fluoroscopy Time	6.2 min
Estimated Blood Loss	20 mL

Initial Device Programming

Mode	DDD
Lower Rate	60 bpm
Upper Tracking Rate	130 bpm
Atrial Sensitivity	0.5 mV
Ventricular Sensitivity	2.5 mV
Atrial Output	2.5 V @ 0.4 ms
Ventricular Output	2.5 V @ 0.4 ms
A-V Delay	180 ms

Procedure Narrative

The patient was brought to the electrophysiology lab in a fasting state. The right pectoral region was prepped and draped in sterile fashion. Local anesthesia with conscious sedation was administered. Venous access was obtained via the right subclavian/axillary vein under ultrasound guidance. An active-fixation atrial lead was advanced and positioned at the right atrial appendage with excellent sensing and pacing thresholds. A ventricular lead was positioned at the right ventricular apex with stable parameters. Leads were secured to the pectoral fascia. A subcutaneous pocket was created in the right pre-pectoral area and the pulse generator connected and placed. Hemostasis was ensured and the pocket irrigated with antibiotic solution. The incision was closed in layers with absorbable sutures and skin adhesive. No immediate complications were observed. The patient tolerated the procedure well and was transferred to recovery in stable condition.

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